

Chemoradiation for anal cancer

Chemoradiation is often the main treatment for anal cancer. This is a combination of radiotherapy and chemotherapy.

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Anal cancer chemoradiation

[Chemoradiation](#) is when you have chemotherapy and radiotherapy at the same time. It is sometimes called chemoradiotherapy. It is often the main treatment for [anal cancer](#).

We have separate information on [radiotherapy](#) and [chemotherapy](#) for anal cancer, which includes detailed information about the individual treatments and their side effects.

Why chemoradiation is given

Chemotherapy drugs can make cancer cells more sensitive to radiotherapy. This can help the radiotherapy to work better. Having both treatments often works better than having either treatment on its own.

Chemoradiation has a better chance of curing anal cancer than surgery. It also means you may avoid [having a stoma](#).

How chemoradiation is given

You usually have chemoradiation as an outpatient. Most people have radiotherapy Monday to Friday for just over 5 weeks. Chemotherapy and radiotherapy start on the same day.

You usually have the chemotherapy drug [mitomycin](#) into a vein on the first day of your radiotherapy only. You then take [capecitabine](#) tablets 2 times a day on the days you have radiotherapy.

Some people have a drug called [fluorouracil](#) into a vein instead of capecitabine. You have it through a portable chemotherapy pump, which you take home. It is usually given continuously for a few days on week 1 and 5 of your radiotherapy.

Your doctor or nurse will tell you about the drugs you will have and how often when you have them.

Before chemoradiation for anal cancer

Before starting this treatment, you should have a blood test to check whether you have low levels of an enzyme called DPD. This is called DPD deficiency.

People who have low DPD levels can develop serious or life-threatening side effects if they have certain chemotherapy drugs. This includes fluorouracil and capecitabine. If you have DPD deficiency, this can affect the treatments that are available for you.

Side effects of chemoradiation for anal cancer

Having [chemotherapy](#) and [radiotherapy](#) together can make the side effects of treatment worse. Your cancer doctor or specialist nurse will give you more information about chemoradiation and the possible side effects.

Some of the common side effects are:

- [tiredness \(fatigue\)](#).
- [diarrhoea](#)
- [sore skin](#) in the area being treated.

Tell your doctor, specialist nurse or radiographer about your side effects. They can help you find ways to cope with them.

During chemoradiation you will usually have weekly blood tests to monitor the levels of your different blood cells.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Below is a sample of the sources used in our anal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

S Rao, MG Guren, K Khan et al. Anal cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. 2021. European Society of Medical Oncology. Available from: www.esmo.org/guidelines/guidelines-by-topic/esmo-clinical-practice-guidelines-gastrointestinal-cancers/anal-cancer [accessed January 2023].

O Dahl, MP Myklebust, JE Dale et al. Evaluation of the stage classification of anal cancer by the TNM 8th version versus the TNM 7th version. 2020. Acta Oncologica, 59(9), 1016–1023. Available from: www.tandfonline.com/doi/full/10.1080/0284186X.2020.1778180 [accessed December 2022].

[Cancer Information Development team](#)

Author

[Dr Paul Ross](#)

Reviewer

Consultant Medical Oncologist

[Guy's and St Thomas' NHS Foundation Trust](#)

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Anal cancer forum

Join our anal cancer forum and chat anonymously to others who understand what you are going through. Our community is available 24/7. It is a safe space where you can share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help